

Psilocibina para trastornos depresivos y de ansiedad en adultos en edad laboral: Protocolo para un análisis bibliométrico híbrido y revisión sistemática con metaanálisis de ensayos controlados aleatorizados (2022 a 2026)

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TÍTULO DE LA REVISIÓN Y DETALLES BÁSICOS

Título de la reseña

Psilocibina para trastornos depresivos y de ansiedad en adultos en edad laboral: Protocolo para un análisis bibliométrico híbrido y revisión sistemática con metaanálisis de ensayos controlados aleatorizados (2022 a 2026)

Condición o dominio que se está estudiando

Depresión; Trastorno depresivo mayor; Trastorno Mixto de Ansiedad y Depresión; Ansiedad; Trastorno de ansiedad; Trastorno de ansiedad generalizada

Trastornos depresivos primarios (incluyendo trastorno depresivo mayor y depresión resistente al tratamiento) y trastornos de ansiedad primarios (incluyendo trastorno de ansiedad generalizada) en adultos en edad laboral (de 18 a 65 años). La depresión o ansiedad secundarias a enfermedades somáticas graves o terminales (como cáncer, enfermedad de Parkinson o demencia) están fuera del alcance de esta revisión, al igual que las condiciones estudiadas sin resultados de depresión o ansiedad reportados por separado (como el trastorno por consumo de alcohol o trastornos alimentarios).

Justificación de la reseña

La depresión y los trastornos de ansiedad representan un problema creciente de salud pública en México. El abandono de los tratamientos psiquiátricos convencionales se asocia a altos costes, efectos secundarios, percepción de mejora prematura y falta de seguimiento profesional, mientras que la percepción de lentitud en la terapia cognitivo-conductual lleva a algunos pacientes a recurrir a fármacos de acción rápida con riesgos potenciales, creando un círculo vicioso que deteriora la funcionalidad y la calidad de vida. La psilocibina, un compuesto psicodélico natural presente en hongos del género *Psilocybe* con un historial de uso ritual y terapéutico en culturas mesoamericanas durante siglos, ha resurgido en la investigación biomédica contemporánea como una alternativa terapéutica experimental. Sin embargo, su rápida transición de fases exploratorias a ensayos controlados aleatorizados de fase 2 y 3 ha superado la capacidad de la evidencia existente para sintetizar. Las revisiones sistemáticas y metaanálisis disponibles agrupan predominantemente ensayos en fases tempranas, combinan poblaciones heterogéneas y mezclan psilocibina con otros psicodélicos, limitando la interpretabilidad clínica de estimaciones agrupadas para pacientes con trastornos depresivos o de ansiedad primarios. Esta revisión sistemática aborda estas limitaciones metodológicas restringiendo la elegibilidad a ensayos controlados aleatorizados publicados entre 2022 y 2026 en adultos de 18 a 65 años con trastornos depresivos y/o de ansiedad primarios, excluyendo depresión o ansiedad secundaria a una enfermedad somática grave. Los análisis de análisis preespecificados y de sensibilidad

examinarán sistemáticamente las fuentes de heterogeneidad. La revisión proporcionará una estimación actualizada y específica para población de la eficacia y seguridad de la psilocibina, con certeza a partir de la evidencia evaluada por GRADE, para informar la investigación clínica y el debate regulatorio durante este periodo de actividad acelerada de ensayos en psiquiatría traslacional.

Objetivos de la revisión

Pregunta principal: En pacientes de 18 a 65 años con trastornos depresivos y/o de ansiedad primarios, ¿el tratamiento con psilocibina, en comparación con placebo o tratamientos habituales, produce mejores resultados clínicos que las alternativas terapéuticas?

Preguntas secundarias:

- (1) ¿Mejora la psilocibina la gravedad, respuesta, remisión, funcionamiento y calidad de vida de los síntomas de ansiedad en comparación con los mismos comparadores?
- (2) ¿Cuál es la frecuencia y el perfil de los eventos adversos asociados a la psilocibina en esta población?
- (3) ¿Varían los efectos del tratamiento dependiendo del régimen de dosificación, la presencia de psicoterapia asistida, la población diagnóstica (trastorno depresivo frente a trastorno de ansiedad) o el tipo de comparador?
- (4) ¿Son robustas las estimaciones agrupadas excluyendo ensayos con alto riesgo de sesgo y preprints?

Infórmate sobre la legislación actual sobre hongos psicoactivos para regular su consumo.

Concienciar y promover el uso responsable de la psilocibina, integrando las tradiciones ancestrales.

Estandarizar los tratamientos convencionales y la psilocibina en cuanto a eficacia, seguridad y modelo biopsicosocial.

Obtener una visión completa y crítica del potencial terapéutico de la psilocibina en comparación con los tratamientos convencionales

Palabras clave

Psilocibina; Terapia asistida con psilocibina; Potencial terapéutico de psilocina; Depresión por psilocibina; Trastorno depresivo mayor; Trastorno Depresivo; Trastornos de ansiedad

País

México

CRITERIOS DE ELEGIBILIDAD

Población

Incluido

Adultos de 18 a 65 años (en edad laboral) con diagnóstico principal de trastorno depresivo (por ejemplo, trastorno depresivo mayor) y/o trastorno de ansiedad (por ejemplo, trastorno de ansiedad generalizada).

Participants required to maintain daily routine and occupational functioning without need for palliative care.

Excluded

Depression or anxiety secondary to severe terminal illness, or explicitly reactive or secondary to somatic pathology such as cancer, Parkinson disease, dementia, or other serious conditions with behavioural implications, including alcoholism and eating disorders.

Intervention(s) or exposure(s)

Included

Psychotherapy; Psilocybin

Psilocybin therapy as the principal intervention for depressive and/or anxiety disorders, with or without assisted psychotherapy. Single or multiple oral doses, defined regimens, with synthetic psilocybin or whole mushrooms.

Excluded

Psilocybin combined with other psychedelics (ayahuasca, MDMA, LSD) or with methamphetamine; interventions centred on conduct disorders, alcoholism or eating disorders without separately reported depression/anxiety outcomes; recreational context.

Comparator(s) or control(s)*Included*

PICO tags selected: Placebo; Antidepressants; Usual Care

Study design

Only randomized study types will be included.

Included

Randomized controlled trials, including phase 2, phase 3 and phase 4 trials, with parallel-group or crossover designs (crossover trials will contribute first-period data where carry-over is plausible given the persistent effects of psilocybin). Trials must report depression and/or anxiety severity on validated scales (for example MADRS, HAM-D, PHQ-9, BDI, HAM-A, GAD-7, STAI, BAI). Peer-reviewed publications and preprints under review (retrieved through Europe PMC, including medRxiv) published from 2022 to 2026 are eligible; preprints that reach journal publication before data extraction will be replaced by the peer-reviewed version. No language restrictions.

Excluded

Systematic reviews and meta-analyses (excluded from the meta-analytic arm only; eligible for the bibliometric arm), phase 1 trials in healthy participants, animal studies, short follow-up case reports, records whose full text cannot be retrieved after institutional access and author contact (documented as "reports not retrieved" in the PRISMA flow).

Context

Trials conducted in any clinical or research setting (inpatient, outpatient, or dedicated research units) and in any country will be eligible; no geographic or income-level restrictions apply. Psilocybin must be administered in a supervised clinical or research context under a defined dosing protocol; studies of recreational, naturalistic, or unsupervised use are excluded. Trials published from 1 January 2022 to the final search date in 2026 are eligible; this window corresponds to the recent expansion of phase 2 and phase 3 trial activity. No language restrictions apply.

SIMILAR REVIEWS**Check for similar records already in PROSPERO**

PROSPERO identified a number of existing PROSPERO records that were similar to this one (last check made on 31 August 2026). These are shown below along with the reasons given by that the review team for the reviews being different and/or proceeding.

- Therapeutic Applications of Psilocybin for Primary and Secondary Depression: A Systematic Review of Randomized Controlled Trials [published 8 May 2026] [CRD420261391152]. The review was acknowledged as **similar** but the authors opted to continue because *there are differences in population, the review looks at additional or different outcomes*

- Therapeutic effects of psilocybin for the treatment of Major Depressive Disorder in adults: a systematic review and meta-analysis [published 31 May 2026] [CRD420261412682]. The review was acknowledged as **similar** but the authors opted to continue because *there are differences in population, the review looks at additional or different outcomes*
- Psilocybin for Depressive Disorders: A Systematic Review and Meta-Analysis of Randomized Controlled Trials [published 5 April 2026] [CRD420261361083]. **The authors did not check this review**
- Safety and Efficacy of Psilocybin-Assisted Psychotherapy in patients with Major Depressive Disorder [published 14 September 2024] [CRD42024556590]. The review was acknowledged as **similar** but the authors opted to continue because *there are differences in population, there are differences in intervention or comparator, the review looks at additional or different outcomes*

TIMELINE OF THE REVIEW

Date of first submission to PROSPERO

This record has not been submitted.

Review timeline

Review start date: 31 August 2026. Review end date: 30 January 2027.

Date of registration in PROSPERO

This record has not been published.

AVAILABILITY OF FULL PROTOCOL

Availability of full protocol

A full protocol has been written but is not available because:

This protocol has undergone several changes

SEARCHING AND SCREENING

Search for unpublished studies

Both published and unpublished studies will be sought.

Main sources that will be searched

The main sources to be searched are *CENTRAL - Cochrane Central Register of Controlled Trials, MEDLINE, PubMed and Scopus*.

Search language restrictions

There are no language restrictions.

Search date restrictions

Databases will be searched for articles published from 1 January 2022, there are no search end date restrictions.

Other methods of identifying studies

Other studies will be identified by: *contacting authors or experts, reference list checking (backward citation searching) and searching trial or study registers*.

Additional information about identifying studies

1. Bibliometric arm (broad strategies; reviews eligible)

- a. Web of Science Core Collection (advanced search)
- b. Scopus
- c. PubMed (bibliometric string)
- 2. Meta-analytic arm
 - a. PubMed
 - b. OVID
 - i. Buscar en todas las revistas de Ovid (Referencias + Abstracts + Revistas Suscritas)
 - ii. EBM Reviews - Cochrane Central Register of Controlled Trials
 - iii. APA PsycInfo <1806 to August 2026 Week 3>

Link to search strategy

A full search strategy has been uploaded to PROSPERO. The PDF may be accessed through this link <https://www.crd.york.ac.uk/PROSPEROFILES/7fddaad2dca577ba1ef4dac746d8afdd.pdf>.

Selection process

Studies will be screened independently by at least two people (or person/machine combination) with a process to resolve differences.

Other relevant information about searching and screening

All searches will be executed on the same day and fully documented following PRISMA-S, recording database, interface, search date and number of records retrieved. Records will be deduplicated in Rayyan or Zotero before screening, matched by DOI and by normalized title. Two reviewers will complete a calibration exercise on a pilot sample of records before formal screening begins, with eligibility criteria refined for clarity (but not scope) if needed. Screening decisions and reasons for full-text exclusion will be documented and reported. Preprints identified through Europe PMC will be checked at data extraction against journal databases; any preprint that has reached peer-reviewed publication will be replaced by the published version, and the substitution will be documented. Retracted articles will be excluded at any stage, with verification against the Retraction Watch database for all included trials before analysis.

DATA COLLECTION PROCESS

Data extraction from published articles and reports

Data will be extracted independently by at least two people (or person/machine combination) with a process to resolve differences.

Authors will be asked to provide any required data not available in published reports.

Study risk of bias or quality assessment

Risk of bias will be assessed using: *Cochrane RoB-2*

Data will be assessed independently by at least two people (or person/machine combination) with a process to resolve differences.

Additional information will **not** be sought from study investigators if required information is unclear or unavailable in the study publications/reports.

Reporting bias assessment

Contour-enhanced funnel plots and Egger's test ($k \geq 10$ per outcome); comparison of reported outcomes against trial registry entries and published protocols to detect selective non-reporting; search-level mitigation

through preprints, trial registries and no language restrictions; publication bias rated as a GRADE domain per outcome.

Certainty assessment

Certainty of evidence will be rated per outcome using the GRADE approach, considering risk of bias, inconsistency, indirectness, imprecision and publication bias. Two reviewers will apply GRADE independently, with disagreements resolved by consensus. Results will be presented in a Summary of Findings table generated with GRADEpro GDT, reporting absolute and relative effects with certainty ratings and footnoted justifications for every downgrade decision. Certainty will be rated separately for the primary outcome (depressive symptom severity) and each secondary outcome (anxiety symptom severity, response, remission, functioning, quality of life, adverse events).

OUTCOMES TO BE ANALYSED

Main outcomes

Primary outcome: change from baseline in depressive symptom severity, measured on validated clinician-rated or self-report scales (MADRS, HAM-D, PHQ-9, BDI or equivalent), at the primary endpoint defined by each trial; where multiple time points are reported, the pre-specified hierarchy will be the trial-defined primary endpoint, then the time point closest to 6 weeks post-dose. Where trials report the same scale, mean differences (MD) will be pooled; across different scales, standardized mean differences (Hedges' g) will be used, both with 95% confidence intervals from random-effects models (REML). Clinician-rated scales will be preferred over self-report when a trial reports both.

Co-primary outcome: change from baseline in anxiety symptom severity on validated scales (HAM-A, GAD-7, STAI, BAI or equivalent), with the same time-point hierarchy and effect measures.

Time points will additionally be grouped for sensitivity analysis as acute (up to 2 weeks), short-term (3 to 8 weeks) and sustained (12 weeks or more) post-dose.

Additional outcomes

Response: proportion of participants achieving at least 50% reduction from baseline in depressive (or anxiety) symptom severity on the primary scale, at the trial-defined primary endpoint. Remission: proportion below the established scale-specific threshold (for example MADRS $\leq$ 10, HAM-D $\leq$ 7, GAD-7 $\leq$ 5). Both will be synthesized as risk ratios (RR) with 95% confidence intervals from random-effects models; risk differences will be reported where absolute effects aid interpretation.

Functioning and quality of life: change from baseline on validated instruments (for example SDS, WSAS, WHOQOL-BREF, SF-36, EQ-5D), synthesized as MD or Hedges' g following the same rules as the primary outcomes.

Safety: proportion of participants with any adverse event, serious adverse events, and adverse events of special interest (treatment-emergent suicidal ideation or behaviour; prolonged perceptual disturbances; acute hypertensive episodes), synthesized as RR where event counts allow, otherwise summarized descriptively with exact counts per arm. Zero-event handling: trials with no events in either arm will be described but not pooled; continuity corrections will not be applied by default, and sensitivity analysis with the Mantel-Haenszel method without correction will be conducted for rare events.

PLANNED DATA SYNTHESIS

Strategy for data synthesis

Random-effects meta-analysis will be conducted for each outcome with at least two contributing comparisons, using REML estimation of between-study variance with the Hartung-Knapp-Sidak-Jonkman adjustment for confidence intervals. Effect measures: MD (same scale) or Hedges' g (different scales) for continuous outcomes; RR for dichotomous outcomes. Heterogeneity will be quantified with I^2 , τ^2 and Cochran's Q, and 95% prediction intervals will be reported for all pooled estimates. Multi-arm trials will contribute the pre-specified primary comparison (least active comparator); if two arms of the same trial are included in one analysis, the shared control group will be split to avoid double counting. Crossover trials will contribute first-period data. Pre-specified moderator analyses (subgroup analysis or random-effects meta-regression as appropriate, conducted when at least 10 comparisons are available): dose regimen, presence of assisted psychotherapy, diagnostic population (depressive versus anxiety disorder), comparator type. Pre-specified sensitivity analyses: exclusion of trials at high risk of bias; exclusion of preprints; time-point grouping (acute, short-term, sustained post-dose); leave-one-out analysis. Any post-hoc analysis will be explicitly labelled as such. Analyses will be performed in R (metafor and meta packages); analysis scripts and derived datasets will be released in the project's public repository. If quantitative synthesis is not feasible for an outcome (fewer than two comparisons or extreme clinical or methodological heterogeneity), results will be reported narratively following SWiM (Synthesis Without Meta-analysis) guidelines, with structured tables and effect direction plots.

CURRENT REVIEW STAGE

Stage of the review at this submission

Review stage	Started	Completed
Pilot work	✓	✓
Formal searching/study identification	✓	
Screening search results against inclusion criteria		
Data extraction or receipt of IPD		
Risk of bias/quality assessment		
Data synthesis		

Review status

The review is currently planned or ongoing.

Publication of review results

Results of the review will be published in English and Spanish.

REVIEW AFFILIATION, FUNDING AND PEER REVIEW

Review team members

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No conflict of interest declared.

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No conflict of interest declared.

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No conflict of interest declared.

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No conflict of interest declared.

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No conflict of interest declared.

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Funding source

Review has no specific/external funding but is supported by guarantor/review team (non-commercial) institutions.

Additional information about funding

No external or specific funding. The review is supported by the authors' academic institution (UNAM) through protected academic time and institutional access of libraries to the full texts.

Peer review

There has been no peer review of this planned review.

ADDITIONAL INFORMATION

Additional information

This review is the meta-analytic arm of a hybrid study whose parallel bibliometric arm has been completed and published, together with the full protocol, in <https://doi.org/10.5281/zenodo.21893380> (GitHub: psilocybin-bibliometrics-2026). The repository includes the PRISMA-P/PRISMA-S protocol, all search strategies, the tri-database deduplication chain, the derived bibliometric corpus, descriptive tables, and figures. The bibliometric arm maps the structure and growth of the field and does not extract clinical outcome data; No screening, extraction, or synthesis of the present review had been initiated at the time of this registration. The analysis scripts, extraction datasets, risk of bias assessments, and GRADE tables from this review will be published in the same public repository upon completion. Amendments to the protocol will be versioned in the repository and reflected in the audit trail of this registry.

Review conflict of interest

Declared individual interests are recorded under team member details.. No additional interests are recorded for this review.

Medical Subject Headings

Adult; Anxiety; Anxiety Disorders; Bibliometrics; Dementia; Depression; Depressive Disorder, Major; Humans; Meta-Analysis as Topic; Middle Aged; Models, Biopsychosocial; Psilocybin; Psychotherapy; Quality of Life; Randomized Controlled Trials as Topic; Psilocybe

PROSPERO version history

No preview available

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